

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date
Sutton, Ron Lewis	22088455	xxx-xx-2822	Male	12/24/37 (88 yrs)
Address	Phone	Email		
424 TAHOE DR CARSON CITY NV 89703-3777	775-882-1662 (H)	sutton.ron1@gmail.com		
Reg Status	PCP	Date Last Verified	Next Review Date	
Verified Registration	Phil M Aldrich, MD775-445-7170	07/17/26	01/13/27	

Sutton, Ron Lewis #22088455 (Acct:8209482219) (88 year old M) PCP: ALDRICH, P (775-445-7170)

01

ED Arrival Information

Expected	Arrival	Acuity	
-	8/9/2026 13:58	Level 4	
Means of arrival	Escorted by	Service	Admission type
Private Transportation	Spouse	Emergency	Emergency

Arrival complaint
Fall; finger/ elbow

Chief Complaint

Complaint	Comment
Fall	
Elbow Injury	
Hand Injury	

ED Treatment Team

Provider	Role	From	To	Phone	Pager
Stephanie M Elmquist, PA-C	Attending Provider	08/09/26 1414	--	775-445-8712	--
Melanie Lugert, RN	Charge Nurse	08/09/26 1414	08/09/26 1414	--	--
Melanie Lugert, RN	Registered Nurse	08/09/26 1414	--	--	--

ED Provider Notes**ED Provider Notes by Stephanie M Elmquist, PA-C**

08/09/26 1643

Author: Stephanie M Elmquist, PA-C Service: — Author Type: Physician Assistant
 Filed: 08/09/26 2158 Date of Service: 08/09/26 1643 Status: Signed
 Editor: Stephanie M Elmquist, PA-C (Physician Assistant)

Emergency Medicine Note:

Patient: Ronald Lewis Sutton; 88 year old male^[SE1]

Chief Complaint

Patient presents with

- Fall
- Elbow Injury
- Hand Injury^[SE2]

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Means of Arrival:[SE1] Private Transportation[SE2]

History of Present Illness

History obtained from[SE1] patient[SE3].[SE1]

Patient is a very pleasant 88-year-old male who's evaluated here in the emergency department for ground level fall, he said he tripped and landed on his hand. He also injured his elbow. He said he did not strike his head. He said when he fell his finger was facing the wrong direction. He said that he grabbed it, he thinks he put it back in place. He said it's still hurting quite a bit, he also has a skin tear to his left elbow. The patient does not take any anticoagulation therapy. He's here today complaining of left elbow and left hand pain. He presents with his wife, he's unsure of his last tetanus immunization.[SE3]

*Nursing and triage documentation reviewed.***Review of Systems:**

All other systems were negative, mentioned in HPI, or non-contributory to this visit[SE1]

Past Medical History:

Diagnosis	Date
• Anxiety	
• Arthritis	
Non	
• Asthma (HCC)	
Non	
• Benign prostatic hyperplasia with lower urinary tract symptoms	
• Bleeding tendency	
Non	
• CAD (coronary artery disease)	
Last seen by Dr. Chryssos 1/25/23	
• Cancer (HCC)	
Non	
• Cardiac defibrillator in place	
• Cardiomyopathy (HCC)	
• Cerebrovascular accident (CVA) (HCC)	
No	
• Convulsions (HCC)	
Non	
• COPD (chronic obstructive pulmonary disease) (HCC)	
• Depression	
• Essential hypertension, benign	
No	
• Gout	
Non	
• History of blood transfusion	

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Non

- Hyperlipidemia
- Myocardial infarction (HCC)
- Obstructive sleep apnea

Does not use cpap (can not tolerate it). dr. McDonald, pulmonary

- Osteoporosis
- ??

- Thyroid disease

No

- TIA (transient ischemic attack)

No

- Type 1 diabetes mellitus (HCC)

Non

- Type 2 diabetes mellitus (HCC)

*Non^[SE2]*PCP:^[SE1] PHIL M ALDRICH

Past Surgical History:

Procedure	Laterality	Date
• APPENDECTOMY		
• AUTOMATIC EXT DEFIBRILLATOR		
• BACK SURGERY		
• CARDIAC SURGERY		
• CATARACT SURGERY		
• CHOLECYSTECTOMY		
• COSMETIC SURGERY		
• FRACTURE SURGERY		
• HERNIA REPAIR		
• ICD GEN CHANGE	N/A	01/19/2022
• INGUINAL HERNIA REPAIR		1988
• JOINT SURGERY		corpral surgery 2024
• LEFT CARPAL TUNNEL RELEASE - Left	Left	05/23/2023
• LYMPH NODE BIOPSY		
• NECK SURGERY		
• TONSILLECTOMY		
• UROLOGY SURGERY	N/A	03/22/2023

Family History

Problem	Relation	Name	Age of Onset
• Heart Disease	Mother	Fiona Sutton	89
• No Family History	Mother	Fiona Sutton	89

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Heart failure

- | | | |
|---------------------|--------|-------------------|
| • Heart Disease | Father | Richard Sutton 89 |
| • Cancer | Father | Richard Sutton 89 |
| • No Family History | Father | Richard Sutton 89 |

Heart failure

- | | | |
|--------------------|---------|------------------------|
| • Cancer, Prostate | Brother | |
| • Cancer, Prostate | Other | Uncle ^[SE2] |

Social History

Social History

Tobacco Use

- | | |
|-----------------------|------------|
| • Smoking status: | Former |
| Current packs/day: | 0.00 |
| Types: | Cigarettes |
| Quit date: | 1980 |
| Years since quitting: | 46.6 |
| • Smokeless tobacco: | Never |

Vaping Use

- | | |
|------------------|------------|
| • Vaping status: | never used |
|------------------|------------|

Substance Use Topics

- | | |
|----------------|------------------------|
| • Alcohol use: | Not Currently |
| • Drug use: | Never ^[SE1] |

Current Outpatient Medications

Medication	Instructions
• ADVAIR DISKUS 250-50 MCG/ACT inhaler	INHALE 1 PUFF INTO THE LUNGS TWICE A DAY FOR 30 DAYS
• albuterol (VENTOLIN HFA) 108 (90 Base) MCG/ACT inhaler	2 puff(s) inhaled every 4 to 6 hours as needed
• aspirin 81 mg enteric coated tablet	1 tablet Orally Once a day
• atorvastatin (LIPITOR) 20 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY
• BREO ELLIPTA 200-25 MCG/ACT aerosol powder, breath activated	1 INHALATION BY MOUTH INTO LUNGS DAILY
• busPIRone (BUSPAR) 5 mg tablet	TAKE 1 TABLET BY MOUTH TWICE A DAY
• cephalexin (KEFLEX)	500 mg, Oral, 4 times daily
• coenzyme Q10 100 mg capsule	1 cap(s)
• cyanocobalamin 500 mcg tablet	1 tablet, Daily
• finasteride (PROSCAR) 5 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY FOR 90 DAYS
• latanoprost (XALATAN) 0.005 % bottle	INSTILL 1 DROP BY OPHTHALMIC ROUTE EVERY EVENING INTO AFFECTED EYE(S)
• losartan (COZAAR)	25 mg, Oral, Daily
• magnesium oxide (MAG-OX) 400 mg tablet	1 tablet

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

- | | |
|--|---------------------------------------|
| • metoprolol succinate ER (TOPROL-XL) 50 mg 24 hr | TAKE 1 TABLET BY MOUTH EVERY DAY |
| sustained release tablet | |
| • Multiple Vitamins-Minerals (MULTIVITAMIN ADULT) tablet | as directed Orally |
| • tamsulosin (FLOMAX) 0.4 MG capsule | TAKE TWO CAPSULES BY MOUTH ONCE DAILY |
| • Turmeric 500 MG tablet | 2 tablets Orally once a day |
| • Vibegron | 75 mg, Oral, Daily |

Outpatient medications were reviewed.^[SE1]

No Known Allergies^[SE2]

Physical Exam:

Vital signs reviewed.^[SE1]

Vitals

	08/09/26 1409	08/09/26 1410
BP:		153/90
Pulse:		77
Temp:		36.9 °C (98.5 °F)
Resp:		22
Height:	167.6 cm (5' 6")	
Weight:	71.3 kg (157 lb 3 oz)	
SpO2:		92%
BMI	25.38 ^[SE2]	
(Calculated):		

167.6 cm (5' 6")

^[SE1] 157 lbs 3.01 oz^[SE2], ^[SE1] Body mass index is 25.37 kg/m².^[SE2]

GENERAL:^[SE1] Well-developed well-nourished 88-year-old male, does not appear to be in distress upon arrival to the emergency department.^[SE3]

HEENT:^[SE1] Head appears normocephalic atraumatic his pupils equal and reactive, conjunctiva clear, scleras nonicteric, oral mucosa moist and pink.^[SE3]

NECK:^[SE1] Neck is supple good range of motion.^[SE3]

HEART:^[SE1] Regular rate and rhythm, no auscultated murmur.^[SE3]

LUNGS:^[SE1] Lungs are clear with good aeration all lung fields.^[SE3]

MUSCULOSKELETAL:^[SE1] The patient is moving his extremities without difficulty aside from his left hand, he has quite a bit of pain in his left ring finger. Patient is able to flex and extend his elbow but he does have some skin tears on the extensor surface. His left finger appears to be slightly dislocated, there is also laceration noted near the PIP joint. Patient has his ring on, I asked him if it's okay if we cut it off and he agreed.^[SE3]

SKIN:^[SE1] 5 mm laceration to the lateral aspect of the left 4th finger near the PIP joint.^[SE3]

NEUROLOGIC:^[SE1] No loss of sensation to his hands.^[SE3]

PSYCHIATRIC:^[SE1] Pleasant and well groomed accompanied by his wife.^[SE3]

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Results:^[SE1]

No results found for this or any previous visit (from the past 48 hours).

XR Fingers 2 or More Views LT

Result Date: 8/9/2026

1. Post reduction of 4th digit PIP dislocation, now with adequate alignment. Electronically Signed by: John Kuipers 8/9/2026 4:32 PM

XR Elbow 2 Views LT

Result Date: 8/9/2026

1. No acute fracture or dislocation. Electronically Signed by: John Kuipers 8/9/2026 2:38 PM

XR Hand 3 or More Views LT

Result Date: 8/9/2026

1. Ulnar sided dislocation at the 4th digit proximal interphalangeal joint. 2. Severe osteoarthritis at the wrist and 1st CMC. 3. There is no discrete acute fracture. Electronically Signed by: John Kuipers 8/9/2026 2:31 PM^[SE2]

ED Course:^[SE1]

ED Course as of 08/09/26 2157

Sun Aug 09, 2026

1444 [XR Hand 3 or More Views LT](#)

IMPRESSION:

1. Ulnar sided dislocation at the 4th digit proximal interphalangeal joint.
2. Severe osteoarthritis at the wrist and 1st CMC.
3. There is no discrete acute fracture.

[SE]

ED Course User Index

[SE] Stephanie M Elmquist, PA-C

Clinical Impressions as of 08/09/26 2157

Dislocation of finger, initial encounter

Laceration of left ring finger without foreign body without damage to nail, initial encounter

Contusion of left elbow, initial encounter

Skin tear of left elbow without complication, initial encounter^[SE2]

Procedures:**Procedures**

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

MEDICAL DECISION MAKING^[SE1]

The patient is evaluated here in the emergency department for his ground level fall, again no head or neck injury. Patient has injury to his hand, certainly would consider fracture versus dislocation versus open fracture, we will also consider fracture versus contusion of his left elbow. Patient will require a tetanus immunization. I will also give him medication for pain

The patient did receive Norco for his pain, Tdap immunization was given, I also ordered lidocaine 1% without epinephrine to do a digital block. The patient's ring was removed by the ER technician. Patient tolerated this well. I then gave the patient a digital block using lidocaine, he had good anesthesia achieved. I was then able to reduce the dislocation of his left 4th finger, dislocation was confirmed on x-ray which showed the following IMPRESSION:

1. Ulnar sided dislocation at the 4th digit proximal interphalangeal joint.
2. Severe osteoarthritis at the wrist and 1st CMC.
3. There is no discrete acute fracture.

Postreduction film shows the following IMPRESSION:

1. Post reduction of 4th digit PIP dislocation, now with adequate alignment.

At this time I told the patient we also needed to repair his laceration, because he had a nice digital block, we did clean the wound thoroughly, I did apply 2 simple interrupted 5-0 nylon sutures to approximate the wound edges. The patient then had a clean bandage placed and a splint was applied. I told the patient I would like him to take the antibiotics as directed, I want him to follow up with the Orthopedics. After his finger was reduced in suture was placed the patient is able to flex and extend at the MCP PIP and DIP joints.

Please note that suture repair was done under sterile technique. Patient tolerated the procedure well. For the digital blocks approximately 3 cc of lidocaine without epinephrine was used

At this time the patient is going to be discharged home, he will certainly return if he has any new or worsening symptoms. The skin tear on the left elbow measured about 2 cm in length, it was cleaned thoroughly and Steri-Strips were applied to hold the flap in place a bandage was applied over the injury^[SE3]

Independent Review of Past Medical Records^[SE1]

Reviewed, patient is currently not on any anticoagulation therapy^[SE3]

ED CONSULTS^[SE1]

None^[SE3]

Social Determinants of Health^[SE1]

Here with his wife who is driving^[SE3]

ED Medications Administered^[SE1]

Tdap immunization, Keflex, lidocaine, Norco^[SE3]

Tobacco cessation counseling^[SE1]

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Not a tobacco user^[SE3]w Meds / Prescriptions:^[SE1]

Discharge Medication List as of 8/9/2026 4:45 PM

START taking these medications

	Details
cephalexin (KEFLEX) 500 mg capsule	Take 1 capsule (500 mg) by mouth 4 times daily for 7 days.Disp-28 capsule, R-0, Normal

CONTINUE these medications which have NOT CHANGED

	Details
Vibegron 75 MG tablet	Take 1 tablet (75 mg) by mouth once daily.Disp-28 tablet, R-0, Sample MedNDC- 73336-075-07 LOT- 122466 EXP- 08/30/2029
losartan (COZAAR) 25 mg tablet	Take 1 tablet (25 mg) by mouth once daily.Disp-90 tablet, R-3, Normal
metoprolol succinate ER (TOPROL-XL) 50 mg 24 hr sustained release tablet	TAKE 1 TABLET BY MOUTH EVERY DAYDisp-90 tablet, R-3, Normal
magnesium oxide (MAG-OX) 400 mg tablet	Take 1 tablet by mouth.Historical Med
busPIRone (BUSPAR) 5 mg tablet	TAKE 1 TABLET BY MOUTH TWICE A DAYDisp-180 tablet, R-3, Normal
ADVAIR DISKUS 250-50 MCG/ ACT inhaler	INHALE 1 PUFF INTO THE LUNGS TWICE A DAY FOR 30 DAYS DAW, Historical Med
finasteride (PROSCAR) 5 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAY FOR 90 DAYS Historical Med
latanoprost (XALATAN) 0.005 % bottle	INSTILL 1 DROP BY OPHTHALMIC ROUTE EVERY EVENING INTO AFFECTED EYE(S) Historical Med
atorvastatin (LIPITOR) 20 mg tablet	TAKE 1 TABLET BY MOUTH EVERY DAYDisp-90 tablet, R-3, Normal
cyanocobalamin 500 mcg tablet	Take 1 tablet by mouth once daily.Historical Med
BREO ELLIPTA 200-25 MCG/ ACT aerosol powder, breath activated	1 INHALATION BY MOUTH INTO LUNGS DAILYDisp-180 each, R-1, Normal

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

albuterol (VENTOLIN HFA) 108 (90 2 puff(s) inhaled every 4 to 6 hours as neededDisp-18 gram, R-3, Normal Base) MCG/ ACT inhaler

aspirin 81 mg enteric coated 1 tablet Orally Once a dayHistorical Med
tablet

coenzyme Q10 100 mg capsule 1 cap(s)Historical Med

Multiple Vitamins-Minerals as directed OrallyHistorical Med
(MULTIVITAMIN ADULT) tablet

tamsulosin (FLOMAX) 0.4 MG TAKE TWO CAPSULES BY MOUTH ONCE DAILYHistorical Med
capsule

Turmeric 500 MG tablet 2 tablets Orally once a dayHistorical Med^[SE2]

Diagnoses:^[SE1]**Final diagnoses:**

Dislocation of finger, initial encounter

Laceration of left ring finger without foreign body without damage to nail, initial encounter

Contusion of left elbow, initial encounter

Skin tear of left elbow without complication, initial encounter^[SE2]**Disposition:**^[SE1]

Discharge

STEPHANIE M ELMQUIST, PA-C^[SE2]

CTEP electronic signature

Portions of this document may have been created by nursing or other staff, these were reviewed and accepted by me. Appropriate corrections were made. This document was created using a computerized dictation system. Although I have reviewed this document, there may still be some phonetic and other typographical errors due to imperfections in the software program and should not be construed in any way to compromise the patient's medical care.

ED Provider Notes (continued)**ED Provider Notes by Stephanie M Elmquist, PA-C (continued)**

08/09/26 1643

Stephanie M Elmquist, PA-C

08/09/26 2158

[SE4]

Attribution Key

SE.1 - Stephanie M Elmquist, PA-C on 08/09/26 1643

SE.2 - Stephanie M Elmquist, PA-C on 08/09/26 2157

SE.3 - Stephanie M Elmquist, PA-C on 08/09/26 2150

SE.4 - Stephanie M Elmquist, PA-C on 08/09/26 2158

ED Course

Value	Comment	By	Time
XR Hand 3 or More Views LT	IMPRESSION: 1. Ulnar sided dislocation at the 4th digit proximal interphalangeal joint. 2. Severe osteoarthritis at the wrist and 1st CMC. 3. There is no discrete acute fracture.	Stephanie M Elmquist, PA-C	08/09 1444

Dictations

None

ED Diagnoses

Diagnosis	Description	Comment	Associated Orders
Final diagnoses			
Dislocation of finger, initial encounter	Dislocation of finger, initial encounter	--	AMB REFERRAL TO ORTHOPEDICS
Laceration of left ring finger without foreign body without damage to nail, initial encounter	Laceration of left ring finger without foreign body without damage to nail, initial encounter	--	AMB REFERRAL TO ORTHOPEDICS
Contusion of left elbow, initial encounter	Contusion of left elbow, initial encounter	--	--
Skin tear of left elbow without complication, initial encounter	Skin tear of left elbow without complication, initial encounter	--	--

ED Disposition

ED Disposition	Condition	Comment
Discharge	--	--

Results

None

Imaging Results

XR Fingers 2 or More Views LT (Final result)

Result time 08/09/26 16:32:50

Final result by John A Kuipers, DO (08/09/26 16:32:50)

Impression:

1. Post reduction of 4th digit PIP dislocation, now with adequate alignment.

Electronically Signed by: John Kuipers 8/9/2026 4:32 PM

Narrative:

EXAM: XR FINGERS 2 OR MORE VIEWS LT

INDICATION: 88 years old, Male. 4th finger post reduction

COMPARISON: Hand radiographs August 9, 2026 at 2:12 PM.

FINDINGS:

Bones: Improved alignment of 4th digit PIP dislocation, now in satisfactory position.

Degenerative Changes: Unchanged severe degenerative changes of the wrist.

Other: Soft tissue swelling of the 4th digit..

XR Hand 3 or More Views LT (Final result)

Result time 08/09/26 14:31:59

Final result by John A Kuipers, DO (08/09/26 14:31:59)

Impression:

1. Ulnar sided dislocation at the 4th digit proximal interphalangeal joint.
2. Severe osteoarthritis at the wrist and 1st CMC.
3. There is no discrete acute fracture.

Electronically Signed by: John Kuipers 8/9/2026 2:31 PM

Narrative:

EXAM: XR HAND 3 OR MORE VIEWS LT

INDICATION: 88 years old, Male. injury

COMPARISON: None.

FINDINGS:

Fracture: No acute fracture. There is ulnar-sided dislocation of the 4th digit middle phalanx relative to the proximal phalanx

Degenerative Changes: Severe degenerative changes throughout the wrist and radiocarpal joint. Severe osteoarthritis with slight subluxation of the 1st CMC joint.

Other: Unremarkable.

XR Elbow 2 Views LT (Final result)

Result time 08/09/26 14:38:53

Final result by John A Kuipers, DO (08/09/26 14:38:53)

Impression:

1. No acute fracture or dislocation.

Electronically Signed by: John Kuipers 8/9/2026 2:38 PM

Narrative:

EXAM: XR ELBOW 2 VIEWS LT

INDICATION: 88 years old, Male. injury

COMPARISON: None.

FINDINGS:

Fracture: No evidence of an acute fracture. No dislocation.

Degenerative Changes: No significant degenerative changes are identified.

Other: No significant effusion.

ECG Results

None

Home Medications

	Taking?	Last Dose	Start Date	End Date	Provider
 ADVAIR DISKUS 250-50 MCG/ACT inhaler	--	--	11/19/23	--	Historical Orders
 albuterol (VENTOLIN HFA) 108 (90 Base) MCG/ACT inhaler 2 puff(s) inhaled every 4 to 6 hours as needed	--	--	01/11/22	--	Phil M Aldrich, MD
 aspirin 81 mg enteric coated tablet	--	--	--	--	Historical Orders
 atorvastatin (LIPITOR) 20 mg tablet TAKE 1 TABLET BY MOUTH EVERY DAY	--	--	01/24/23	--	Phil M Aldrich, MD
 BREO ELLIPTA 200-25 MCG/ACT aerosol powder, breath activated 1 INHALATION BY MOUTH INTO LUNGS DAILY	--	--	11/06/22	--	Phil M Aldrich, MD
 busPIRone (BUSPAR) 5 mg tablet TAKE 1 TABLET BY MOUTH TWICE A DAY Patient not taking: Reported on 4/13/2026	--	--	02/01/24	--	Phil M Aldrich, MD
 coenzyme Q10 100 mg capsule	--	--	--	--	Historical Orders
 cyanocobalamin 500 mcg tablet	--	--	--	--	Historical Orders
 finasteride (PROSCAR) 5 mg tablet	--	--	--	--	Historical Orders
 latanoprost (XALATAN) 0.005 % bottle	--	--	11/14/23	--	Historical Orders
 losartan (COZAAR) 25 mg tablet Take 1 tablet (25 mg) by mouth once daily.	--	--	09/11/25	--	Basil Emanuel Chryssos, MD
 magnesium oxide (MAG-OX) 400 mg tablet	--	--	07/02/24	--	Historical Orders

	Taking?	Last Dose	Start Date	End Date	Provider
 metoprolol succinate ER (TOPROL-XL) 50 mg 24 hr sustained release tablet TAKE 1 TABLET BY MOUTH EVERY DAY		--	09/10/25	--	Basil Emanuel Chryssos, MD
 Multiple Vitamins-Minerals (MULTIVITAMIN ADULT) tablet		--	--	--	Historical Orders
 tamsulosin (FLOMAX) 0.4 MG capsule		--	09/01/20	--	Historical Orders
 Turmeric 500 MG tablet		--	--	--	Historical Orders
 Vibegron 75 MG tablet		--	01/26/26	01/26/27	Jeffrey Dwight Upton, MD

Take 1 tablet (75 mg) by mouth once daily.
Patient not taking: Reported on 4/13/2026
Notes: NDC- 73336-075-07
LOT- 122466
EXP- 08/30/2029

Medication Administration from 08/09/2026 1358 to 08/09/2026 1707

Date/Time	Order	Dose	Route	Action	Action by	Comments
08/09/2026 1650 PDT	lidocaine (XYLOCAINE) 1 % injection 10 mL	10 mL	Subcutaneous	Given by Other	Melanie Lugert, RN	--
08/09/2026 1507 PDT	HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet	1 tablet	Oral	Given	Melanie Lugert, RN	--
08/09/2026 1700 PDT	tetanus-diphth-acell pertussis (BOOSTRIX) injection 0.5 mL	0.5 mL	Intramuscular	Given	Melanie Lugert, RN	--
08/09/2026 1650 PDT	cephalexin (KEFLEX) capsule 500 mg	500 mg	Oral	Given	Melanie Lugert, RN	--

ED Prescriptions

Medication	Sig	Dispense	Start Date	End Date	Auth. Provider
cephalexin (KEFLEX) 500 mg capsule	Take 1 capsule (500 mg) by mouth 4 times daily for 7 days.	28 capsule	8/9/2026	8/16/2026	Stephanie M Elmquist, PA-C

ED Medication Orders (From admission, onward)

Start	Ordered		Status	Ordering Provider
08/09/26 1700	08/09/26 1629	tetanus-diphth-acell pertussis (BOOSTRIX) injection 0.5 mL Once Route: Intramuscular Dose: 0.5 mL	Last MAR action: Given	ELMQUIST, STEPHANIE M
08/09/26 1700	08/09/26 1629	cephalexin (KEFLEX) capsule 500 mg Once Route: Oral Ordered Dose: 500 mg	Last MAR action: Given	ELMQUIST, STEPHANIE M
08/09/26 1500	08/09/26 1449	HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet Once	Last MAR action: Given	ELMQUIST, STEPHANIE M

Start	Ordered	Status	Ordering Provider
		Route: Oral Ordered Dose: 1 tablet	
08/09/26 1445	08/09/26 1433	lidocaine (XYLOCAINE) 1 % injection 10 mL Once Route: Subcutaneous Ordered Dose: 10 mL	Last MAR action: Given by Other ELMQUIST, STEPHANIE M

Code, Iso, Restraint (From admission, onward)

None

ED Imaging Orders (From admission, onward)

Start	Ordered	Status	Ordering Provider
08/09/26 1608	08/09/26 1607	XR Fingers 2 or More Views LT 1 time imaging Final result	ELMQUIST, STEPHANIE M
08/09/26 1413	08/09/26 1412	XR Elbow 2 Views LT 1 time imaging Comments: Look for fracture/injury. Final result	HENDERSON, TIMOTHY KOPLIK
08/09/26 1412	08/09/26 1412	XR Hand 3 or More Views LT 1 time imaging Comments: Look for fracture/injury. Final result	HENDERSON, TIMOTHY KOPLIK

ED Micro, Lab, POCT (From admission, onward)

None

ED All Orders (From admission, onward)

Start	Ordered	Status	Ordering Provider
08/09/26 1630	08/09/26 1629	Finger Splint Continuous Canceled	ELMQUIST, STEPHANIE M
08/09/26 1608	08/09/26 1607	XR Fingers 2 or More Views LT 1 time imaging Final result	ELMQUIST, STEPHANIE M
08/09/26 1413	08/09/26 1412	XR Elbow 2 Views LT 1 time imaging Comments: Look for fracture/injury. Final result	HENDERSON, TIMOTHY KOPLIK
08/09/26 1412	08/09/26 1412	XR Hand 3 or More Views LT 1 time imaging Comments: Look for fracture/injury. Final result	HENDERSON, TIMOTHY KOPLIK
08/09/26 0000	08/09/26 1644	AMB Referral to Orthopedics Comments: 4th left finger dislocation Specialty: ORTHOPAEDICS Provider: (Not yet assigned) Ordered	ELMQUIST, STEPHANIE M

Discharge Orders (From admission, onward)

Start	Ordered	Status	Ordering User
08/09/26 0000	08/09/26 1629	cephalexin (KEFLEX) 500 mg capsule 4 times daily Ordered	ELMQUIST, STEPHANIE
08/09/26 0000	08/09/26 1644	AMB Referral to Orthopedics Comments: 4th left finger dislocation Specialty: ORTHOPAEDICS Provider: (Not yet assigned) Ordered	ELMQUIST, STEPHANIE

Allergies (Reviewed on: 08/09/26)

No Known Allergies

Immunizations Administered

Name

Tdap

Tetanus Up To Date

None

Medical History

Diagnosis	Date	Comment	Source
Anxiety			
Arthritis		Non	
Asthma (HCC)		Non	
Benign prostatic hyperplasia with lower urinary tract symptoms			
Bleeding tendency		Non	
CAD (coronary artery disease)		Last seen by Dr. Chryssos 1/25/23	
Cancer (HCC)		Non	
Cardiac defibrillator in place			
Cardiomyopathy (HCC)			
Cerebrovascular accident (CVA) (HCC)		No	
Convulsions (HCC)		Non	
COPD (chronic obstructive pulmonary disease) (HCC)			
Depression			
Essential hypertension, benign		No	
Gout		Non	
History of blood transfusion		Non	
Hyperlipidemia			
Myocardial infarction (HCC)			
Obstructive sleep apnea		Does not use cpap (can not tolerate it). dr. McDonald, pulmonary	
Osteoporosis		??	
Thyroid disease		No	
TIA (transient ischemic attack)		No	
Type 1 diabetes mellitus (HCC)		Non	
Type 2 diabetes mellitus (HCC)		Non	

Surgical History

Procedure	Laterality	Date	Comment	Source
APPENDECTOMY			No	
AUTOMATIC EXT DEFIBRILLATOR				
BACK SURGERY			No	
CARDIAC SURGERY			No	
CATARACT SURGERY				
CHOLECYSTECTOMY			No	
COSMETIC SURGERY			No	
FRACTURE SURGERY			No	
HERNIA REPAIR				
ICD GEN CHANGE	N/A	01/19/2022	Procedure: EP ICD Gen Change;	

Surgical History (continued)

Procedure	Laterality	Date	Comment	Source
			Surgeon: Jeffrey Turner, DO; Location: CTH RMC Cardiac Cath Lab; Service: Electrophysiology; Laterality: N/A;	
INGUINAL HERNIA REPAIR		1988		
JOINT SURGERY		corpral surgery 2024		
LEFT CARPAL TUNNEL RELEASE - Left	Left	05/23/2023		
LYMPH NODE BIOPSY			No	
NECK SURGERY			No	
TONSILLECTOMY				
UROLOGY SURGERY	N/A	03/22/2023	CYSTOSCOPY, TRANSURETHRAL RESECTION OF PROSTATE	

Social History

Tobacco History

Smoking Status **Quit Date** **Smoking Tobacco Type**
Former **1980** **Cigarettes quit in 1980**

Pack Year History

Packs/Day	From	To	Years
0	1980		46.6

Smokeless Tobacco Use
Never

Alcohol History

Alcohol Use Status
Not Currently

Drug Use

Drug Use Status
Never

Sexual Activity

Sexually Active
Not Asked

Other Factors

Not Asked

Social Documentation

No social documentation on file.

Family Medical History

Problem	Relation	Age of Onset	Comments
Cancer	Father		
Cancer, Prostate	Brother		
Cancer, Prostate	Other		
Heart Disease	Father		
Heart Disease	Mother		
No Family History	Father		Heart failure
No Family History	Mother		Heart failure

ED LDA Documentation (8/8/2026 00:00 to 8/9/2026 17:07)

ED Events

Date/Time	Event	User	Comments
08/09/26 1358	Patient arrived in ED	MUNOZ MARQUEZ, GUADALUPE	--
08/09/26 1358	Emergency encounter created	MUNOZ MARQUEZ, GUADALUPE	--
08/09/26 1407	Triage Started	ENBODY, CASSANDRA	--
08/09/26 1409	Triage Completed	ENBODY, CASSANDRA	--
08/09/26 1414	Assign Attending	ELMQUIST, STEPHANIE	Stephanie M Elmquist, PA-C assigned as Attending
08/09/26 1414	Assign Physician	ELMQUIST, STEPHANIE	--
08/09/26 1414	Patient roomed in ED	LUGERT, MELANIE	To room VC02
08/09/26 1414	Team Member Removed	LUGERT, MELANIE	Melanie Lugert, RN removed as Charge Nurse
08/09/26 1414	Assign Nurse	LUGERT, MELANIE	Melanie Lugert, RN assigned as Registered Nurse
08/09/26 1414	Patient transferred	LUGERT, MELANIE	--
08/09/26 1421	Registration Completed	MATA, MONICA	--
08/09/26 1707	ED Tracking End	LUGERT, MELANIE	--
08/09/26 1707	Patient discharged	LUGERT, MELANIE	--

Follow-up Information

Follow up With	Specialties	Details	Why	Contact Info
Carson Tahoe Emergency Department	Emergency Medicine		If symptoms worsen	1600 Medical Parkway Carson City Nevada 89703 775-445-8733
Carson Tahoe Emergency Department	Emergency Medicine	In 10 days	For suture removal	1600 Medical Parkway Carson City Nevada 89703 775-445-8733
Phil M Aldrich, MD	INTERNAL MEDICINE GENERAL	Schedule an appointment as soon as possible for a visit		2874 N CARSON ST STE 200 Carson City NV 89706 775-445-7170
RENO ORTHOPEDIC CLINIC		Schedule an appointment as soon as possible for a visit		1365 Medical Pkwy Carson City Nevada 89703 775-786-3040
RENO ORTHOPEDIC CLINIC				1365 Medical Pkwy Carson City Nevada 89703 775-786-3040

Discharge Instructions

Please keep bandage on for 2 days and then wash daily with soap and water. Suture removal in 10 days.

Please follow with Orthopedics, I did write for antibiotics because you had a laceration overlying your proximal interphalangeal joint. Please return if you have any new or worsening symptoms

Discharge References/Attachments

Dislocation, Finger (English)
Elbow Bruise (English)
Laceration, All Closures (English)

AVS Reports

Date/Time	Report	Action	User
8/9/2026 4:45 PM	ED AVS (After Visit Summary)	Printed	Stephanie M Elmquist, PA-C

Communication Routing History

Recipient	Method	Sent by	Date Sent
RENO ORTHOPEDIC CLINIC Fax: 775-785-3437 Phone: 775-786-3040	Fax	Stephanie M Elmquist, PA-C	8/10/2026

ED Charges

Description	Code	Date	Service Prov	Modifiers	Qty	Status
HB DISLOCATION CLSD INTERPHALANGEAL JNT, W/MANIPULATION W/OUT ANESTHESIA	7002298	08/09/2026	Elmquist, Stephanie M	F8	1	Filed
HB REPAIR SIMPLE HEAD/NK/EXT/TRNK/GEN/EXTR <2.5 CM	7002362	08/09/2026	Elmquist, Stephanie M		1	Filed
HB IM/SC INJECTION	7003200	08/09/2026			1	Filed
HB EMERGENCY DEPT VISIT 4	6000008	08/09/2026	Lugert, Melanie	25	1	Filed
HB IMMUNIZATION ADMINISTRATION ONE VACCINE	7002802	08/09/2026	Elmquist, Stephanie M		1	Filed
HB WILLOW REV 636 GY	5040093	08/09/2026	Elmquist, Stephanie M		1	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	08/09/2026	Elmquist, Stephanie M		1	Filed
HB WILLOW REV 636	5040098	08/09/2026	Elmquist, Stephanie M		200	Filed
HB WILLOW REV 250 CARE 637 GY	5040066	08/09/2026	Elmquist, Stephanie M		1	Filed